

جوستاف روسي انترناسيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : عزيزه صلاح عباس محمد
المحافظة : الجيزة
الحى / المدينة : امبابه
رقم البطاقة : 26912090102246 رقم التليفون : 01273078436
العمر(تاريخ الميلاد) : 09-12-1969 المهنة : بدون عمل
العنوان : م
تاريخ تسجيل الإستمارة : 28-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام

----- التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة:

الوظيفة:

الوظيفة:

التوقع:

التوقع:

التوقيع:

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس

وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

قسم :

نَفَقَات :

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: عزيزه صلاح عباس محمد National ID: 26912090102246	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

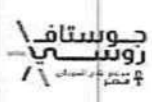
نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

 جوسناف رؤس مصر 9-٢٢		مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية	السلطة المصرية صحة مصر
			09/22/2026
MDT FORM			
Name: عزيزه مصطفى عباس محمد		MRN: 13477	
Age: 57 Y		ID: 26912090102246	
Menopausal Sta: Post		Phone:* 01131037158	
NRS SCORE:		Marital Status: Married	
BMI:		Family History:	
Gender:			
CO Morbidity: DM, HTN, ISHDs (stents)		Reason Of MDT:	
		NACT	
Surgical History:* -		ECOG Performance Status	
		© 0 1 2 3 4	

Clinical Data:

BX: IDC G3
ER: 0/8 **PR:** 2/8 **HER2:** 0 **KI67:** 50%

History:

Bilateral breast masses:

Left breast biopsy: Mastitis (abcess)

Rt breast mass : IDC (5/2026)

SM 14/6/2026: UIQ lesion 10*13*11 mm + ipsilateral LN

Referred for NACT

Path revision: IDC G3, ER 0/8 PR 2/8 HER2 0 KI67: 50%

CT CAP: 30/7/2026: Rt breast mass + Rt axillary LN 1.2 cm

Bone scan: 7/2026: free

For new CT CAP + SM

Sonomammography:

O/E: skin edema + nipple retraction

Biomarker/Molecular**Mets Workup:****Recommendations:****MDT Outcome & Plan**

new CT CAP + SM

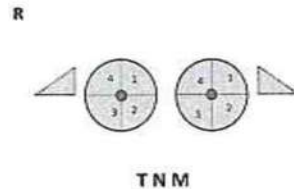
BRCA test

Portacath insertion

NACT: Paclitaxel - Carboplatin x 12 weeks + Pembrolizumab then 4 AC x

4 cycles + pembrolizumab

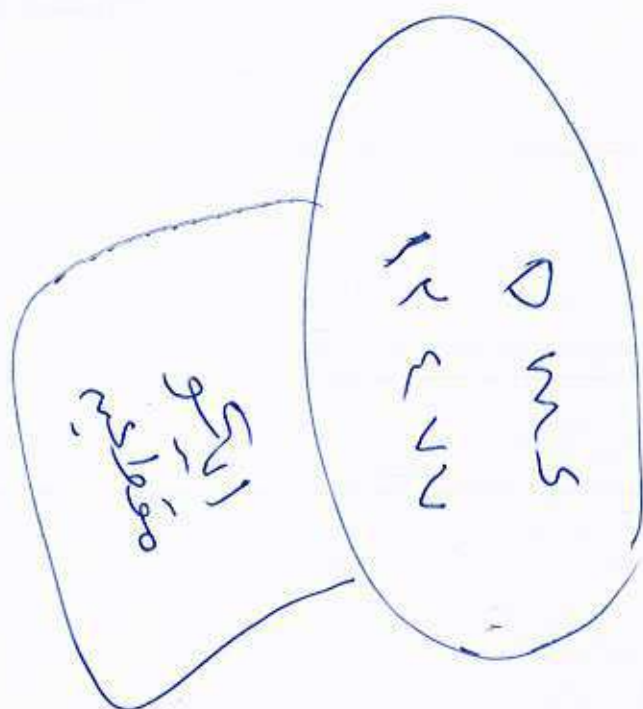
Then Rt MRM



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:**Pathological Staging:****SIGNATURE**

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د خالد عبد العزيز	د محمد كبراي		





Patient Name	عزيزه صلاح عباس	Case No.	360777
Sex/Age	Female 56 Y	Patient No.	26396
Registered Date	26/8/2026 02:58:12 PM	Referred By Dr.	Out Patient
Validation Date	15/9/2026 01:03:43 PM	Branch	Gustave Roussy
Reporting Date	19/9/2026 11:42:07 AM	Pathology No.	SR-984/26

Pathology Report

Clinical Data: Right breast cancer.

Nature of Specimen: Block revision.

Gross Examination:

- One referred paraffin block labeled by (956/H) was received.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- **Histologic type:** Invasive duct carcinoma
- **Histologic grade:** 3 (differentiation 3, pleomorphism 3, mitosis 2)
- **DCIS:** Absent
- **Microcalcification:** Absent
- **Angioinvasion:** Absent
- **Perineural invasion:** Absent
- **Tumor infiltrating lymphocytes (TILs):** Moderate

Results:

* IPT (All controls showed appropriate reactivity):
- ER: No nuclear reaction in tumor cells (score 0/8).
- PR: Weak nuclear reaction in <1% of tumor cells (score 2/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67%: 50%.

Conclusion:

Breast mass, Slide revision:

- Invasive duct carcinoma, Grade 3.

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 2/8).



Patient Name: AZIZA SALAH ABAS AZIZA	Age: 56 Years
Patient ID: 26912090102246	Study Date: 22/04/2026 12:41:26

01154651421

Bilateral breasts Sonomammography examination

Mammography revealed :

- ACR :B scattered glandular pattern
- Right breast UOQ illdefined dense lesion with surrounding architectural distortion.
- No suspicious calcifications seen.
- Normal nipple areolar complex.

The complementary ultrasound shows

- Both breasts show fibroglandular fatty tissue.
- The right breast shows an illdefined irregular hypoechoic spiculated hypoechoic lesion measuring about 10x8x5.6mm seen in the UIQ at 11 o'clock 4cm from the nipple with low vascularity.
- The left breast shows no suspicious masses or cysts.
- No dilated ducts seen.
- Normal nipple areolar complex.
- Bilateral axillary non specific lymph nodes, with preserved hilum.

Conclusion:

Right breast UOQ suspicious lesion as described (BIRADS IV) for further biopsy and histopathological assessment.

for Dr. Iryny Salib Biopsy
23/4

Best Regards,

Dr: Iryny Salib

Iryny Salib

Age: 55	Study Date: 14/06/2026 10:58:20
Patient Name English: AZIZA SALAH	

Bilateral breasts Sonography examination

- **Known case of pathologically proven right breast cancer**
- **Both breasts show fibroglandular fatty tissue.**
- **The right breast :**
Still noticed ill defined irregular hypoechoic spiculated hypoechoic lesion measuring now about 10x13x11 mm compared to 10x8x5.6mm in previous 2 months ago study seen in the UIQ at 11 o'clock 4cm from the nipple with low vascularity.ipsilateral axilla show lymph node with focal cortical thickening reaching 3.5 mm.
- **The left breast : showing cutaneous and subcutaneous rather well defined heterogenous inflammatory lesions seen measuring about 3.6x2.2 cm in diameter showing surrounding interstitial edema as well as skin thickening seen at 12 O'clock measuring about 3.6x2.2 cm in diameter.ipsilateral reactive lymph node seen.**
- **Normal nipple areolar complex.**

Conclusion:

Known case of right breast cancer showing

- Mild progressive course regarding size of Right breast UOQ malignant lesion (BIRADS 6) with ipsilateral pathological lymph node.

-Left breast upper central superficial abcess (BIRADS III) for surgical management.

Dr. Aya Hassan,MD

Patient Name: عزيزه صلاح عباس محمد
MRN: 10210126131545
D.O.B: 09/12/1969
IC No.: 26912090102246

Location:
Sex:

External Clinic
Female

Surgical Pathology

Specimen

Specimen Id 2606201213
Accession No SU-26-3917
Sample Type TISSUE

Collection Date 20/06/2026 10:17 AM
Received Date 20/06/2026 10:17 AM

Specimen

Left breast abscess wall biopsy.

GROSS DESCRIPTION

A single ring-shaped skin covered tissue piece measuring 4x3x1cm with unremarkable greyish rubbery cut section. Multisected and totally submitted.

Blocks:

1. Superior margin.
2. Inferior Margin.
- 3,4. Medial Margin.
- 5,6. Lateral Margin.

MICROSCOPIC

Examination of serial sections reveal tissue fragments covered by stratified squamous epithelium.

The dermis is infiltrated by acute and chronic inflammatory cells mainly neutrophils, lymphocytes, plasma cells and histocytes.

Areas of granulation tissue formed of collagen, fibroblast, newly formed capillaries and mixed inflammatory cells are also seen.

No evidence of malignancy.

DIAGNOSIS

Left breast abscess wall biopsy: - ✓
- Mastitis with abscess formation.
- No malignancy.

Signature

Prof. Dr. Manal Fawzy
22/6/2026

Revised by

Soha Yehya
Esraa Essam

Reported and electronically endorsed by
Anatomical Pathology Unit

Validated on 24/06/2026 12:17 PM



Patient Name: عزيزه صلاح عباس	Patient ID: 503023
Study Date: 30/07/2026 09:20:00	

CT scan of the Chest , abdomen & Pelvis :-

A known case of right breast cancer

Chest :

- Right breast dense parenchyma with ill defined nodules
- Right axillary LN 1.2cm
- Clear both lungs with no definite focal lesions, infiltrations or bronchiectatic changes.
- No pleural effusion
- Normal cardiac size with no pericardial effusion.
- No gross mediastinal masses or lymphadenopathy are detected.

Abdomen & Pelvis

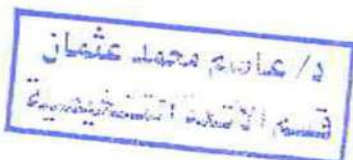
- Mild hepatomegaly with no focal lesion
- Normal CT appearance of the spleen, adrenal glands, pancreas, kidneys, IVC and aorta.
- No definite abdominal or pelvic lymphadenopathy.
- No ascites.
- Normal pelvic organs.

typist

PROF: DR MOHAMED ABD ELLATEF.

HEND

Spelling checked by



خطابات أرقام

Patient Name : Azizah Salah Abbas

Date: 25/7/2026

Referring physician: Prof.

^{99m}Tc-MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- No scintigraphic evidence of bony active osseous lesions .
- Heterogenous abnormal radiotracer uptake is seen at the rest of the spine , more determined at , mid dorsal , upper lumbar vertebrae , likely of Spondylodegenerative changes .
- Marked osteoarthritic changes are seen at , both shoulders , both knees joints
- Both kidneys are normally seen.

Conclusion:

No definite hot spot that could account for osseous metastatic lesions

Thanks for the referral

Nuclear Medicine Unit

DR . Sherin Mahdy, MD

DR/ Mohamed Emam

د/ محمد أميم
٢٠٢٦/٧/٢٥

جوسناف
روستاق
في مستشفى جوسناف



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الهيئة العامة
لصحة
المرأة

09/22/2026

MDT FORM

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Mets Workup:

Recommendations:

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new CT CAP + SM
BRCA test
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NACT: Paclitaxel - Carboplatin x 12 weeks + Pembrolizumab then 4 AC x 4 cycles + pembrolizumab
Then Rt MRM

TNM

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. خالد عبد الوهيد	د. محمد توفيق		

about:blank



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Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي - مصر - مستشفى البشير لوجي



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Reporting Date	19/9/2026 11:42:07 AM	Pathology No.	SR-984/26

- Her2: Negative (score 0).
- Ki67%: 50%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%, 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim. MD